



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : عليه سلام مصطفى سليمان
المحافظة : القاهرة الحي / المدينة : دار السلام
رقم البطاقة : 25701252100809 رقم التليفون : 01284563920
العمر (تاريخ الميلاد) : 25-01-1957 المهنة : ربه منزل
العنوان : دار السلام
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: عليه سلام مصطفى سليمان National ID: 25701252100809	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مقتضية



 جمهورية مصر العربية وزارة الصحة 07/21/2025	
MDT FORM	
Name: Alyia Salama Mostafa AB	HRN: 5735
Age: 70 Y	ID: 25701252100809
Menopausal Sta: postm	Phone: 01127111720
MRS SCORE:	Marital Status:
BMI:	Family History:
Gender:	Reason Of MDT:
CO Morbidity:	MBC HR +VE TO BONE LIVER LUNG WITH DP IN OSSEOUS LESIONS
Surgical History: DE NOVO MBC	ECOG Performance Status 0 1 2 3 4

Clinical Data:

DX: ICC
 ER: 8/8 ER: 5/8 HER2: -VE KIT67:



TNM

History:

postm rt breast cancer metastatic to bone liver qutry lung nr8/6 pr 7/8
 her2-va diagnosed 2022 rec al 6/2024 dp st fasoldeix Rance BR
 11/2025 dp st afinitor aromisin cts on 9/3/2025 newly developed h8 seg
 4 pd of prev h8 largest in 3x2.5 seg 6 newly developed osseous lesion d4
 6 10

PT REC 4 CYCLES XELODA

bone scan on 20/6 no comparison no prev bs in 2025
 widespread osseous lesions in mid and lower dorsal vertebrae heavy
 metastatic deposits
 CTS 10/6 NEWLY DEVELOPED D9 COLLAPSE SD OF BILORAR HFL
 LARGEST M 18X12

SD OF PULMONARY NODULES M 9X7MM FOR NDT

Screening mammography

Biopsy/Molecular

Mdx Workup

Recommendations

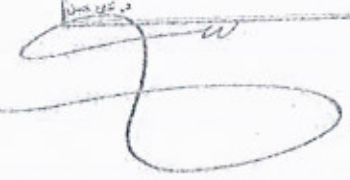
MDT Outcome & Plan

In view of the Pt being 70 yo with several bone mets (difficult mobility and
 preferred Oral TK) also after reviewing CTs: patient has overall SD with
 only new D9 structural collapse.
 So, to Continue Xeloda and Zometa /3m
 -Palliative RT4 to D9 (after ortho consultation if eligible for fraction)

Clinical Staging

Pathological Staging

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
			

Name : Aliy Salam Mustafa Saleiman

Date : 20-06-2026 10:49

Branch : Maadi

Medical No. : 397999

Ref.Physician Prof : Gonstav Roussey Egypt

Ins.Company : عقدة مستشفى

Whole-Body Bone Scintigraphy Report

Clinical History

A 69-year-old female with advanced right breast malignancy with liver metastases; presents with generalized bone pain. No history of bone surgery, trauma, chemotherapy, or radiotherapy. Re-evaluation for skeletal metastasis; no prior studies for comparison.

Technique

Routine whole-body anterior and posterior planar images were acquired 3 hours following the intravenous administration of Tc-99m MDP.

Findings

Axial Skeleton: Markedly increased heterogeneous uptake in the skull vault, prominent posteriorly. Active focal uptake in the sternal body. Spinal column shows intense uptake, with heavy, dense tracer accumulation concentrated in the mid and lower dorsal spine. Pelvic uptake is patchy throughout right sacroiliac articulation.

Appendicular Skeleton: Benign-looking marked degenerative changes are noted at weight-bearing joints as well as both shoulders with no suspicious pattern.

Soft Tissue & Excretion: Normal soft tissue background activity is noted, with tracer excretion seen within the kidneys and urinary bladder.

Impression

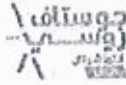
Widespread, intensely increased heterogeneous osteoblastic activity as described.

Given the clinical context of advanced breast malignancy, these findings are highly characteristic of diffuse skeletal metastases.

Mid and lower dorsal spine: Demonstrate heavy metastatic deposits; clinical and radiation oncology correlation is recommended for palliative radiotherapy if justified.

Dr. Alaa Nagi, MD
Nuclear Medicine

Call Center 19656



Patient's Name: Ali Salama Mustafa Suleiman
Date: 2026-06-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

As compared to previous study dated 9/3/2026, the current study revealed

I- CHEST:

- o Still seen right breast heterogenous parenchyma with skin thickening and right axillary mass lesion is seen measuring 33 x 31 mm for sono-mammography.
- o Still seen few bilobar pulmonary nodules largest measuring 9 x 7 mm at the apical segment of the right lower lobe.
- o Small consolidation is seen at the medial segment of the left lower lobe.
- o No pulmonary masses or cavitary lesions.
- o Still seen few mediastinal lymph nodes largest measuring 12 x 11 mm at the retrocaval region.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. Stationary course of the few bilobar hypodense hepatic focal lesions largest measuring 18 x 12 mm at segment VI. No dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o Few subcentimetric bilateral external iliac lymph nodes.
- o No ascites.
- o Still seen left hypogastric nodule measures 1cm
- o Newly developed D9 vertebral structural collapse with evidence of retropulsion
- o Still seen D7 structural collapse
- o still seen D6 and D8 vertebral wedging noted.



Patient's Name: Ali Salama Mustafa Suleiman
Date: 2026-06-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Opinion:

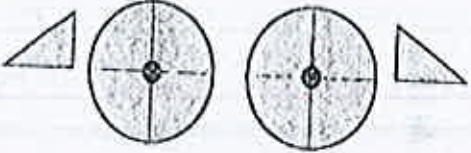
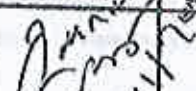
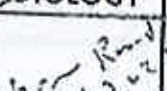
- ❖ Newly developed D9 vertebral collapse with posterior retropulsion
- ❖ Stationary rest of study

Dr.sherif Wagdy

Dr.sherif Wagdy

MDT FORM

19/ 11 / 20 25 G

Name		Ullorh Sile		MRN	6099 / 1
Age		68		ID	COV-1000-1
Menopausal status				Phone	101807498
NRS SCORE				Marital status	
BMI				Family history	
CO morbidity				REASON OF MDT	
Clinical Data					
BX:				R	
ER:	PR:	HER 2:	Ki67:	L	
6/8	7A	-ve			
Sonomammography:				TNM	
Mets Workup:					
Met Breast to Loc) received AJ → OP					
Recommendations:					
switched to doxorubicin 6/10/24 ↑ SC 2/11/2020 → increase size N to HQ Nov 2024 with 1.2x14 clinical progress					
MDT OUTCOME & PLAN:					
for Adjuvant 1 Proven Shift to 20h. Adjuvant 1 Proven					
SIGNATURE:					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
 9/11/24	 10/11/24	 11/11/24			

لجنة
2022/6/13
2022/6/13

Patient Name : الأمانة / عليه السلام مصطفى بالتماني
Patient No. : 25701252100809
Path No : 1627/2022
Sex : Female
Age : 65 Y
Referring Doc. :

Request No : 13000131
Request Date : 13/06/2022
Verification Date : 04/07/2022
Financial Category : أورام دار السلام - عائلة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass and left pathological axillary lymph node.

Nature of the specimen
Tru cut biopsy, collected at 12/6/2022, 01:40 PM.

Gross Examination:

Two specimens were received

1. Right breast:

Multiple grayish white and fatty tissue cores up to 2 cm, totally submitted.

2. Left axillary lymph node:

Fragmented grayish white and hemorrhagic tissue cores measured collectively 1x1cm, totally submitted.

Immunohistochemical technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2. ER clone (SP1), PR clone (IE2), HER-2 CLONE (4B5) & Ki67 (30-9).

Microscopic examination:

1. Right breast mass:

Histological type: Invasive duct carcinoma

Grade (differentiation + nuclear pleomorphism + mitosis): II (3+2+1)

Angiolymphovascular invasion: Negative

Ductal carcinoma in situ: No

2. Left axillary lymph nodes:

Positive for metastatic deposits of duct carcinoma.

Verified by:

DR. Menna Nabil

Printing Date: 28/09/2022 09:50:18

Page 1 of 2

Printed by: marwa mohamed yousse

Patient Name: الأمانة / عليا محمد مصطفى سليمان
Patient No.: 25701252100809
Path No.: 1627/2022
Sex: Female
Age: 65 Y
Referring Doc.:

Request No: 13000131
Request Date: 13/06/2022
Verification Date: 04/07/2022
Financial Category: أورام داخل المستشفى - نفاذ الدولة
Department: Out Patient

Registered by: manwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- IPT of the right breast mass and left axillary lymph nodes
 - Examination of ER stained slide show intense nuclear staining in 100% of the tumor cells (both)
 - Examination of PR stained slide show intense nuclear staining in 80% of the tumor cells (both)
 - Examination of HER2 stained slide show no staining in the tumor cells (both)
 - Examination of Ki67 stained slide show nuclear staining in 25% of tumor cells (both).
- Positive tissue control showed appropriate reactivity

Diagnosis

- Right breast mass & pathologically enlarged left ALNs; Tru cut biopsy
- Right breast biopsy with invasive duct carcinoma, grade II.
- Left axillary lymph nodes with positive metastatic deposits of duct carcinoma.
- IPT of the right breast mass and left axillary lymph nodes:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- HER-2: Negative (score 0).
- Ki67: Positive in 25% of tumor cells.

Prof. Dr. Gamal Abd Alatty
Dr. Walaa Ghonaim

Verified by:
DR. Menna Nabil

Printing Date: 28/09/2022 09:50:18

Page 2 of 2

Related By: manwa mohamed yousse

Patient Name: Aleya Sallam Moustafa Soliman	Date : 21/03/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 66 Year(s)
Medical ID: 2599/2022	Pat. ID: 14974

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular parenchyma (ACR B)
- Right edema pattern (Increased skin thickening and coarse trabeculation)
- Right breast large well defined dense mass lesion with suspicious grouped of microcalcifications, roughly measuring about 10x6 cm (In CC view).

Complementary ultrasound :

- Right Increased skin thickening with dilated subdermal lymphatics
- Right barest globally large heterogenous lesion copying almost all the breast parenchyma associated with nipple retraction
 - Another smaller mass seen at 10o'clock zone C , measured about 26x23 mm seen inseparable from the underlying muscle
 - 7o'clock (Inframammary fold) skin nodule , measured about 3x0.5cm
- No left cystic or solid mass lesions.
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Multiple right axillary enlarged LNs with diffuse cortical thickening and effaced fatty hilum seen at level I with Q extra capsular invasion., the largest one measuring about 13x8 mm.
- free supra-clavicular node LN.
- left multiple enlarged axillary LNS with effaced hilum , the largest measured about 23x13mm

Opinion:

- right locally advanced pathologically proved carcinoma with bilateral pathological axillary LNS (BIRADS 6).
- Right skin nodule
- Left barest fibro adenosis BIRADS 2.

Signature Dr. Walaa Abdelazez

21/03/2023 12:01 PM